COTIVITI



The payment integrity playbook

for success in 2023 and beyond

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The state of payment integrity in 2023

Although healthcare payments have undergone major transformations over the past decade, one thing is constant—the need for claims payment integrity remains paramount for payers. Not only does every inappropriately overpaid claim represent a cost that takes away from member care, these errors also erode the payer-provider-member relationship.

Health plans have bolstered their ability to prevent inappropriate claim payments before they're paid through advanced analytics and new pause-and-review programs in recent years, but postpay solutions continue to remain vital, coupled with optimizing coordination of benefits in tandem with the member lifecycle; addressing E&M over-coding, preventing inappropriate specialty drug spending; and managing fraud, waste, and abuse (FWA).

This is your guide to payment integrity in 2023. This eBook will offer you a solid foundation for holistic payment success across your critical programs.

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Prepay claims editing: Execute more accurately with less provider abrasion

Claim editing is generally a health plan's first line of defense within its prospective payment integrity program. It's a practice based on following guidelines and rules from organizations like the Centers for Medicare & Medicaid Services (CMS) and the American Medical Association (AMA) and codifying them into claim editing rules. How hard can it be to stand up a strong claim editing system? The answer may be surprising.

Some payers develop homegrown claim editing while many others follow the advice of organizations like Gartner, investing in vendor-supplied solutions to alleviate dependency on internal IT. But whether building or buying, health plans must understand and be prepared to handle claim editing challenges or be sure that their vendor solution can.

Recency, compliance, accuracy, and transparency of claim editing is of the utmost importance to a successful claim editing solution. One cannot just "set and forget" claim editing software. It requires constant maintenance and oversight for the highest accuracy and transparency, and there are many resources needed to adequately maintain it. Payers should consider several best practices when deploying a claim editing solution that pays claims with the highest accuracy and the lowest provider abrasion.

Extensive clinical resources

Broad and diverse clinical resources are needed to help you achieve and maintain the highest levels of payment integrity. And the more variety the better: certified

Recency, compliance, accuracy, and transparency of claim editing is of the utmost importance to a successful claim editing solution.

coders, specialized registered nurses, and medical directors have different perspectives and areas of expertise when it comes to researching, building, and maintaining editing policies for regulatory requirements from AMA, CMS, state Medicaid agencies, and other bodies.

Regulatory alignment

When industry sources release updates to code sets (such as HCPCS, CPT, and ICD-10), teams must be prepared to review all new and deleted codes for a clear picture of how changes affect policies. This includes staying abreast of the timeline for these updates, as changes need to be made before the effective dates of the new code sets.

For example, evolving National and Local Coverage Determinations (NCD/LCD), which often change without notice, require constant review to stay current. Similarly, every state in which a plan has licensed Medicaid business has different payment rules and documentation practices that need to be constantly monitored for change to avoid falling behind on payment integrity practices.

Technical resources and support

Technical resources are also key to updating the claim editing application—especially when there's a regulatory update or policy change. And there's no time to wait in a technical request queue, as delays can cause improper payments and noncompliance. Claim editing applications must reflect software updates released by vendors of integrated claim processing systems, while a dedicated technical staff must translate policies into properly functioning claim edits and maintain those edits for every policy and/or regulatory update. To update systems per these changes, payers may need to hire more technical staff.

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Accurate claim editing also depends on a high level of agility. Most policies will change multiple times throughout the year, and a payer must be willing and able to handle that volume of change. To put this need for agility in perspective, it is not uncommon for a health plan to need to update as much as 96% of its policies at least once a year—with many updated twice or more annually. This could easily require 10 quarterly/semi-annual industry change reviews (e.g., CPT, HCPCS, ICD) and monthly reference reviews to help maintain the amount of avoided overpayment savings accrued for these policies.

When a payer implements a vendor's software system, it must ensure the vendor is prepared to handle such changes without exorbitant maintenance fees. The vendor should also support the client in optimizing data exchange, including implementation of new data elements to support new payment policy content.

Analytic and reporting resources

A health plan building its own or maintaining third-party claim editing software should also employ analytic and reporting staffers to provide leadership with ad-hoc reports and claim spend insights, as well as a strong understanding of provider trends for proactive policy enhancement.

For example, a plan may use analytics and reporting resources to identify potential over-coding rates among certain providers for evaluation and management (E&M) claims, where a level 4 or 5 was coded on the claim unnecessarily. Analysts should be able to prepare reports showing leadership the potential losses from physician office E&M policies not being followed by outlier physicians. With information in hand and leadership approval, the plan would be able to immediately turn on the right editing rule for a specific line of business, targeting only the highest outlier physician offices to start with to minimize abrasion.

E&M over-coding: A proven approach to prevention without abrasion

Given the high claim volumes and relatively low unit dollars involved, Evaluation and Management (E&M) claims present a significant payment integrity challenge for payers, as the cost of postpay audit and recovery often outweighs the inflated costs of over-coding, not to mention high provider abrasion.

For professional office E&M claims, Cotiviti's own analysis of more than 45 payers, including 9 of the 10 largest plans in the nation, demonstrates overpayments occur approximately 25% of the time on average, with some outlier providers over-coding E&M claims 50% or more of the time. These over-coded claims amount to millions of dollars of overpayments annually for health plans. For emergency room (ER) facility E&M claims, Cotiviti's analysis found that Level 4 and 5 claims rose by more than 32% from 2016 to 2021 while Level 1, 2, and 3 claims decreased by 23%.

Read our white paper by Cotiviti Chief Medical Officer Richard Pozen, M.D., as he explains how an approach that combines machine learning and advanced analytics with expert nurse coder review delivers significant savings for payers—and enables them to pay even the most complex E&M claims promptly and appropriately while minimizing the potential for provider pushback.

Read the white paper

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Decreased provider abrasion

By creating a fast claim editing process, payers increase cost-effectiveness and decrease the provider abrasion that results from delaying claims payment or recovering overpayments. By implementing an automated claims editing system, supplemented with all the necessary human experts mentioned above, payers gain a payment integrity solution that is constantly and reliably updated for accuracy and regulatory changes, ultimately satisfying most providers.

Another component for decreasing provider abrasion is being transparent with providers about existing updates and new claim editing policies. The user experience for both plans and providers looking into the details of particular claim decisions is an important component of claim editing. For example, a user-friendly system and process that allows claim inquiries to be answered quickly and with full, defensible rationale by claim analysts, while extending that same rationale through the plan's web portal, gives providers the convenience of self-service. Together, these factors can help cut out lengthy processes that can frustrate both payers and providers.

Broad and diverse clinical resources are needed to help you achieve and maintain the highest levels of payment integrity.

Provider abrasion can also be decreased by health plans understanding the relative provider "acceptability" rate of new edits before deploying them, no matter the level of defensibility of the source. For example, knowing that 75% of providers in a health plan's region are exposed to the same edits with very low appeal rates would give the payer much less pause to deploy them.

A reasonable solution

Though claim editing can look simple from afar, its many moving parts make it a complex, constantly changing machine. However, it doesn't need to be overwhelming, and it doesn't need to increase provider abrasion.

Optimal prospective payment integrity solutions require diverse components to function with speed, efficiency, and transparency. Whether your chosen route is internally made or outsourced, ensure that its total cost of ownership is worth it. When choosing to go your own way or to go with a managed service, be sure that teams have the capacity to research and develop edits, ensure system interoperability, and perform constant maintenance. If partnering with a vendor, an independently owned solutions and services provider that has nationwide experience with both payers and physicians offers the greatest breadth and depth of experience, ensuring that your payment integrity solutions can scale as your organization grows.

Taking action

One regional Blue Plan serving more than 4 million members discovered that improper claims were passing through its homegrown claim editing system, causing the plan significant financial leakage. Aiming to reduce medical costs and shore up prepay claim accuracy, the plan implemented Cotiviti's Payment Policy Management solution as a secondary editing system to provide final filter editing. After realizing significant savings, the plan added Cotiviti's Coding Validation solution, a final prepay review of more clinically complex claims by nurses and coding experts.

Read our case study and see how the plan achieved nearly $100 million in annual savings, including:

- $62M professional claim savings
- $36M outpatient claim savings
- $24.75 PMPY savings total

Read the case study

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Postpay claim accuracy: Why retrospective solutions are still vital

Prospective payment integrity solutions continue to gain traction among health plans to lower medical costs and improve relationships with providers. However, prepay solutions are only one tool available to ensure payment integrity. The foundational value of retrospective solutions, particularly for complex claims, makes them a vital part of any successful enterprise payment integrity program.

When it comes to payment integrity, prospective solutions are lauded for how much they accomplish before payment. And indeed, prepay solutions help speed the payment process so that claims can be verified faster. But prepay solutions are based on an existing high level of confidence in the accuracy of the claim information, which is most effective for less complex, routine claims. Postpay solutions are necessary to handle complex claims that require deeper scrutiny for verification. Claim complexity increases as data, validation, and timing requirements rise—and the more complex the claim is, the more plans need to rely on retrospective solutions.

Here are a few scenarios where retrospective payment integrity solutions are preferred and even necessary.

Additional scrutiny for complex claims

Although payment for many claims can be automated, complex claims benefit from an added level of examination. In fact, part of the benefit of postpayment integrity solutions is that they can catch more problematic claims than prepay solutions can, whether due to the technological limits of a plan's system, prepay time constraints, or claims excluded from the prepay data set: our analysis shows that 74% of the overpaid claims we identify with our postpayment data mining tools were excluded from the prepayment data set provided by our client.

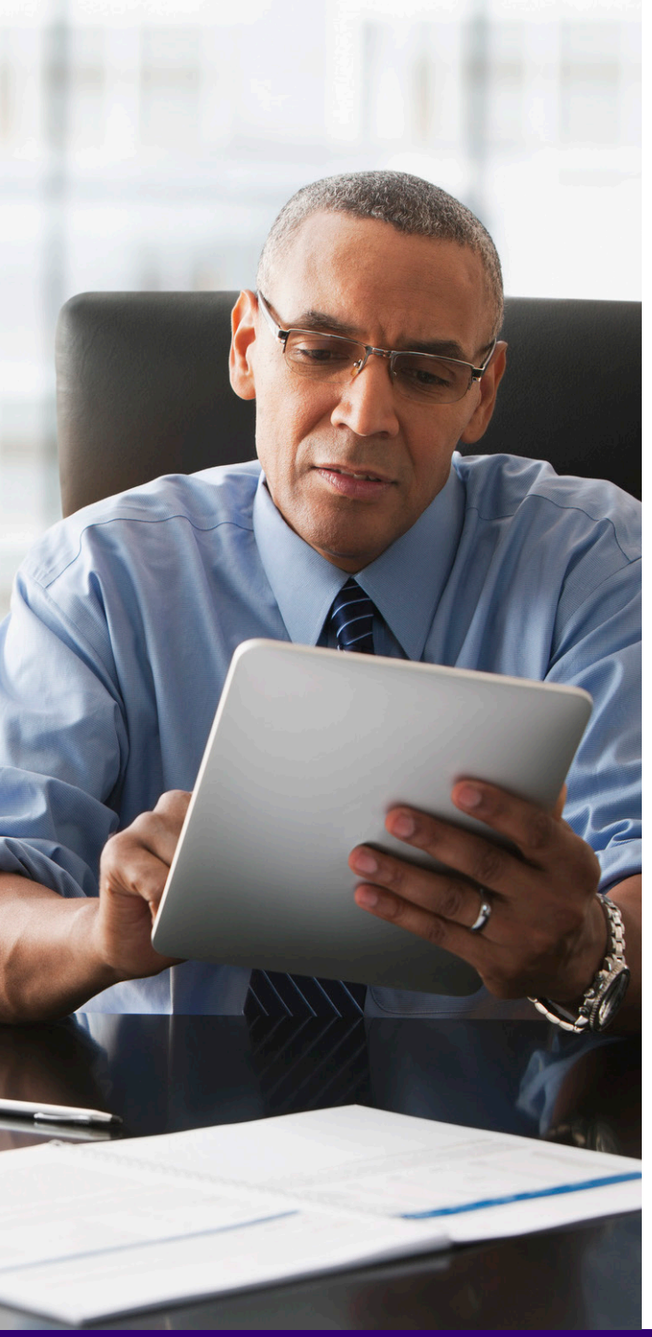
The human intervention component of a postpay solution can perform additional outreach needed to verify claims manually, especially in cases of adjudication errors.

Simpler claims align to pre-approved policies and require only claim data—and possibly a quick look at medical records—for validation, but more complex claims need additional data or provider outreach and can benefit from going through a postpay data mining solution. This is because postpay data mining solutions can catch issues such as complex incorrect modifiers, procedures, or revenue code mappings; multiple claim payments; or corrected claim modifications that prepay solutions may not capture.

Human intervention for outreach and judgment

Claims that are even more intricate require specialized, human expertise to accurately analyze. After a thorough cycle of automated data mining to flag possible incorrect claims, human intervention from certified experts, beyond the interventions performed in prepay solutions, offers an in-depth, white-glove touch to claim editing. The human intervention component of a postpay solution can perform additional outreach needed to verify claims manually, especially in cases of adjudication errors. Human intervention can identify billing errors that need to be corrected, invoice or billing reviews, complex unit issues, or simply wrong information, such as claim edits that target incorrect providers or services that automated prepay processes would not identify. Other examples include contract-based reviews to ensure the intent of all terms,

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service-level definitions, and controls to ensure that rate hierarchies are applied correctly.

Clarity for claims triggered by future events

Postpay solutions are an imperative for claims that will be triggered by events that haven't yet occurred. From late charges to retroactive fee schedule changes, there are many scenarios requiring a complete and thorough review of services over multiple claims and dates of service. Retroactive terminations are a good example of this. As dictated by the Affordable Care Act, retroactive terminations are only allowed in specific scenarios, and require a second look to verify. They therefore must be verified after the fact.

Other examples of increased complexity involve scenarios such as:

- Claim-level duplicates
- Corrected claims
- Retroactive contract pricing issues
- Medicare/other insurance primacy confirmation
- Drug frequency
- Adjudication errors

Contract compliance review

Contractual terms, prompt pay laws, and other state regulations all form the foundations on which healthcare claims should be quickly and accurately reimbursed per your providers' agreements. To add another layer of complexity, payers must keep data safe and follow regulatory mandates such as HIPAA, HITECH, and SOC. Postpay claim reviews, supported by advanced analytics and data mining tools, must be deployed to remain compliant with relevant regulation and contracted policy.

Using postpay solutions in concert with prepay analytics is the key to a well-rounded payment integrity strategy. Whether as an added verification measure, a need for additional human intervention, or to navigate multilayered complexities, plans should consider what balance of postpay and prepay intervention work for them.

Taking action

One mid-sized health plan with nearly 1 million total members across primarily commercial and ASO lines of business partnered with Cotiviti to help close payment integrity gaps, focusing mostly on correct-coding concepts. The plan deployed Cotiviti's COB Validation and Data Mining solutions to audit and recover overpayments, and soon after implemented Payment Policy Management. Later, the payer added Clinical Chart Validation to audit and recover improper inpatient DRG payments postpayment, followed by Coding Validation for professional claim review.

Read our case study and see how the plan achieved:

- $19.5M annual savings
- $14 PMPY prepay savings
- $6 PMPY postpay savings

Read the case study

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Coordination of benefits: Keeping members in mind

Coordination of benefits (COB) requires health plans to navigate significant complexities. The order of benefit determination rules rely on an ever-changing member landscape that is as variable and unique as the members themselves. It can be easy, therefore, to forget to put the claims lifecycle in context against the actual member experience—getting a new job, getting married or divorced, starting a family, or retirement. These are just a few life events relevant to COB that should be considered during the claims adjudication and payment process.

While there is no simple, single solution to simplifying COB, designing a robust COB process that considers the entire member experience is a significant step forward. Why? Because it can:

- Help capture an additional 60-70% of COB opportunity when compared to looking solely at the claim lifecycle
- Set the stage for early intervention where possible—assessing COB before a claim is paid, or even before it is received or treatment is rendered
- Provide the opportunity to integrate prospective and retrospective payment integrity processes into a streamlined process with varying intervention points
- Improve the overall member experience, limit provider abrasion, and accelerate speed to value

Identifying COB members for review

While identifying members in need of a COB review can be somewhat daunting, plans can start with the data on hand. Entitlement and eligibility information from CMS and the Council for Affordable Quality Healthcare (CAQH) can provide some valuable data points to fuel your investigation. Machine learning algorithms can also speed up this process, helping find members with both dual entitlement and other primary coverage.

By designing a COB process predicated on the lifecycle of your members, rather than just claims, you can address and resolve payment issues earlier—often before claims are received or paid.

A word of caution: while these data sources can be extremely valuable, these information repositories are not standalone solutions. Rather, they can serve as an instrument in your broader COB program and should be used in conjunction with other tools and strategies. For populations and data points outside of available in-house assets, a rich COB validation plan goes back to the member lifecycle, reviewing members that present with specific data points and events that tie to COB relevant life events.

For example, let's take a family with two parents and three dependent children. The eldest dependent child is turning 26 and will no longer be eligible for coverage under their parents as a dependent. It doesn't cost the parents anything additional to keep their oldest child on the plan. And there don't appear to be any claims for the oldest child in the past four months after a period of continuous treatment. This trend could indicate that this young adult got a job that now offers health insurance, which overlaps with prior treatment, for which your health plan was the primary payer. It's certainly worth exploring.

Finding the other insurance

The next thing to do is to identify the second insurance. Investigating whether someone has Medicare entitlement is more of a straightforward process as there is only one source to contact, whether through a CMS representative or a web-based portal.

Trying to identify a commercial payer or payers can be a little more challenging. First,

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plans must understand who the payers are in the state and/or region where the contract was issued, and then communicate with them for additional information. Another method to identify the other payer is to work with the provider when investigating gaps in coverage and routine claims. For example: Cotiviti observed a member that had been receiving routine injections. After a year, the claims for the provider stopped for four months and then picked up again. We spoke to the provider, who confirmed that during the lapse, they had been receiving payment from another payer. With this new information, we were able to work with the other payer to establish primacy order.

Determining the order of benefits

Once you've selected your member population and identified the other payers, the next step is to make the order of benefit determination. To complete this, examine the member's data profile to ensure that the information is the most current needed to make the order of benefit determination.

The employment status of the subscriber, for example, is one critical item to verify. You may need to perform an employment verification, as oftentimes working status updates are only captured during annual open enrollment time frames. Employment verifications can also be a great source for information pertaining to group size, which is another data element that may be needed to perform the order of benefits determination.

When reaching out to employers, you want to make sure you are asking the right questions and tailoring the outreach method to your employer group preferences (such as whether they prefer emails, spreadsheets, or even a shared data repository).

The bottom line: by designing a COB process predicated on the lifecycle of your members, rather than just claims, you can address and resolve payment issues earlier—often before claims are received or paid.

Taking action

Watch our panel webinar as Cotiviti payment leaders walk through specific examples demonstrating how plans can account for variable member life events to help ensure proper COB determinations by:

- Looking beyond claims to see the whole member experience
- Performing focused and intentional member selection
- Using predictive analytics and expert experience to address complex billing scenarios
- Integrating prospective and retrospective payment integrity processes

Watch the webinar



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Fraud, waste, and abuse: 3 trends to watch for in 2023

As the health system bent under the weight of the COVID-19 pandemic, bad actors used this opportunity to devise schemes to extract money from the system through FWA. While their schemes largely utilized the same tactics as in previous years, the extraordinary growth in telehealth created new opportunities as well. We've outlined three common schemes special investigative units (SIUs) should expect to see persist this year and beyond—and how to combat them.

FWA in behavioral telehealth claims

Since the COVID-19 pandemic began, telehealth has been a key factor in treating behavioral health issues. According to Cotiviti's data, the volume contribution for behavioral health procedure codes in telehealth shifted from 0.5% in 2019 to 40% in 2022. One popular scheme is to bill plans for individual therapy when they should be billed for group therapy. Cotiviti's own data found a specific case where 57% of one provider's encounters included billing for individual psychotherapy visits for multiple members of the same household. After a record review, Cotiviti experts confirmed that the 60-minute individual therapy sessions should have been billed as group therapy, resulting in overpayments to this provider totaling >$500K. Another common example is billing for "impossible days," as seen when one provider billed for 26 hours and eight patients per date of service on average.

Unbundling services

Unbundling schemes, which remain persistent in healthcare, are another example of when prepay analytics are highly useful. Providers may bill separately for overlapping services or section out a single procedure into multiple claims. To expose unbundling, pay attention to high utilization of modifiers in your prepay editing system and the corresponding services. One particular example Cotiviti observed was a provider utilizing modifier XU to bill multiple levels of molecular pathology procedures, as well as an unlisted molecular pathology procedure for the same patient on the same date of service.

Excessive testing and upcoding

Outlier analysis remains a core technique for SIUs to spot billing for unnecessary services. Cotiviti noticed a provider billing for more than 20 unique genetic tests per member—well over double the average of the provider's peers. Another provider had a very high ratio of COVID-19 tests billed compared to peers, alerting investigators to dig into more patterns that uncovered other concerns.

Excessive testing is often accompanied by upcoding. One provider Cotiviti investigated not only billed a definitive drug test for every single patient who received a presumptive drug test, which is unusual, but billed 77% of these definitive drug tests at the highest possible level. For comparison, the provider's peers billed at this level only 11% of the time.

Tips for FWA success in 2023

Many of the classic FWA schemes investigators have observed for years have not changed, allowing you to leverage knowledge to follow the data and use internal mechanisms to drive more successful FWA prevention. Ensure you collaborate with other stakeholders across the payment integrity continuum so you are never working in a silo. Working together will promote innovation within your organization and improve your investigations. Finally, pay attention to emerging trends via prepay analytics to help predict future schemes and identify current ones. All of these efforts will promote a holistic approach to combatting FWA and protect our healthcare system.

Taking action

Learn how Health Partners Plans of Pennsylvania successfully recovered more than $1 million in overpayments related to upcoding from a provider group in partnership with Cotiviti.

Read the case study

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Specialty drugs: Going beyond prior authorizations

Specialty drugs comprise a complex, high-stakes area of claim processing for health plans. These medications are often injectable drugs administered by a provider and are used to treat chronic, complex diseases such as cancer, cystic fibrosis, HIV, atopic dermatitis, and autoimmune disorders. Specialty drugs are notoriously expensive—and growing—with specialty pharmaceutical pricing growing by an estimated 4–6% in 2022 and new drugs being approved every month.

Relying solely on prior authorizations to deliver prospective payment integrity can leave room for significant financial losses and mean that valuable data slips through the cracks. And the complexity behind specialty drug claims means that nuances can often be lost in any automation that is not designed to catch common errors.

Here are the key attributes of a successful specialty drug payment integrity program:

- **Dedicate a team to drug-specific payment integrity solutions.** Do not short-change the solution by lumping it in with other, broad-sweeping tools that only look at the "big picture." A pharmacist is an obvious must-have, but a well-rounded team that also includes pharmacy technicians, nurses, physicians, and data analysts is ideal to provide a complete perspective on specialty drugs billed to the medical and pharmacy benefits.
- **Focus on the details.** Drug claims contain countless details from NDC to quantity to diagnosis and so much more. Each detail has value but also presents an opportunity for errors—and corrections.
- **Approach from multiple angles and points of view.** Prepay solutions can prevent errors and waste while maintaining good provider relationships, but a postpay safety net solution is also necessary for scenarios that require additional verification/confirmation.
- **Create a closed loop system.** Connecting prepay, postpay, and FWA programs and teams creates a process that covers all areas. For example, connecting postpay claim review results to your FWA pattern detection analytics can reduce duplication of effort and speed up the final conclusion, as well as feed other initiatives like machine-learning driven insights and accuracy.

Taking action

Dive deeper into specialty drug payment integrity challenges with our white paper by Cotiviti's Jonathan Starr, PharmD, who outlines best practices for handling specialty drug claims in a way that goes beyond prior authorization for better accuracy and faster payment.

Read the white paper

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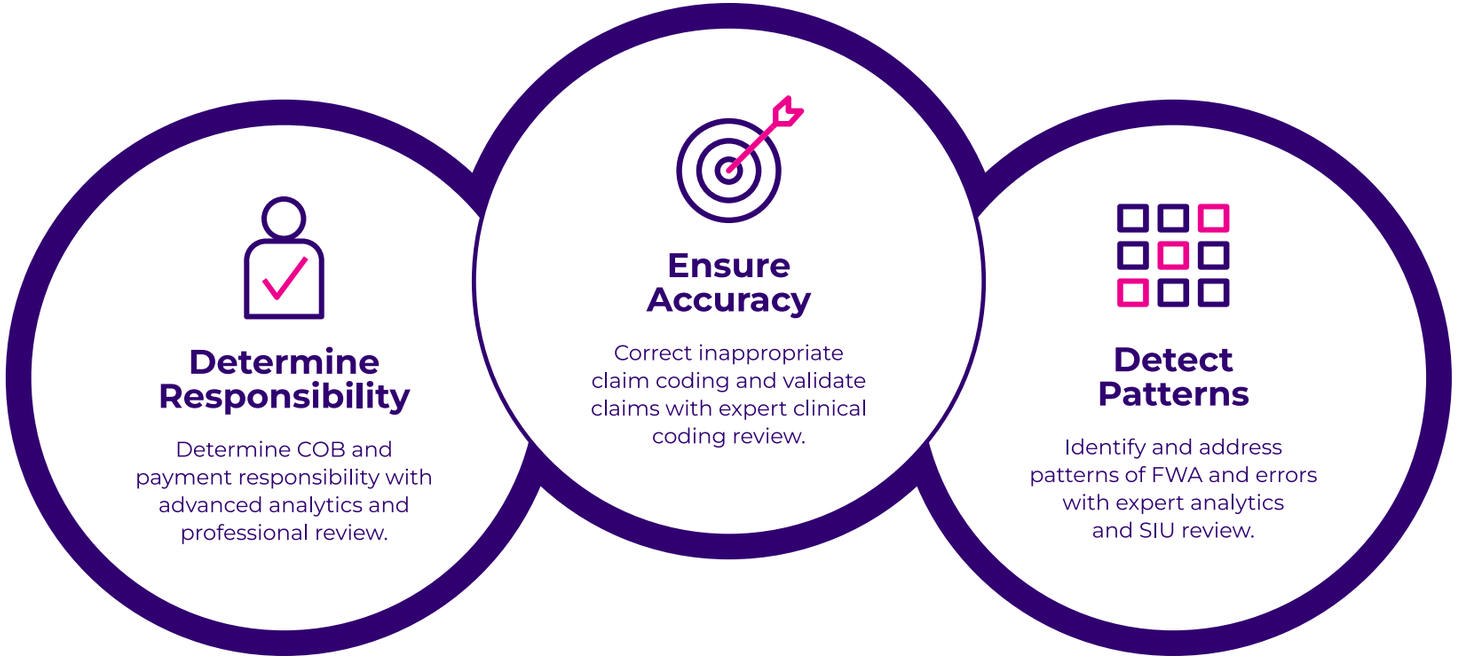
Bringing it all together

By using the programs, solutions, and strategies outlined in our eBook, you can increase savings, improve provider relations, and optimize member interactions with your organization.

Key lessons from this guide:

- Follow best practices when deploying a prepay claim editing solution
- Don't underestimate the value of postpay data mining
- Design a holistic COB process that considers the entire member experience
- Be vigilant to catch longstanding FWA schemes resurfacing
- Build internal expertise to prevent costly overpayments for specialty drugs

We hope this guide empowers you to take the necessary steps towards transforming your payment integrity programs in 2023 and beyond.



Achieve better value with Cotiviti

As health plan claim volumes grow amid cost increases and changing guidelines, the opportunity for inappropriate claims to slip through the cracks and get paid grows, too. Especially when plans use a siloed and disconnected approach to payment integrity.

Cotiviti's Payment Accuracy suite helps you knock down those silos and perform beyond time pressure by shortening time-to-value from more than 90 days to less than five. With dynamic solutions that can work together to review 100% of all major claim types at the right intervention point, our clients can flexibly manage their MLR and unlock more incremental savings value, with less burden on provider partners and administrative teams. In fact, we've helped our clients achieve more than $7 billion in annual payment integrity savings through our scale, expertise, integration, and unparalleled accuracy.

Taking action

Learn how Cotiviti can help you achieve better enterprise value and request a one-on-one demo at www.cotiviti.com.

Learn more

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